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PSYCHOLOGICAL ASSESSMENT REPORT

SOCIODEMOGRAPHIC DATA

NAME: Mr. Rahul Ahirwar

Age: 19 years

GENDER: Male

Date of Birth: 26/12/2006

EDUCATION: 12th Std. ongoing

RELIGION: Hindu

MARITAL STATUS: Unmarried

OPD NO.: 221219008

SOCIO-ECONOMIC STATUS: Middle

AADHAR NO.: 486582314008

ADDRESS: Ruthiyai, M.P

INFORMANTS DETAILS

NAME	RELATIONSHIP	DURATION
1. Mr. HS	Father	19 years

Reliability and Adequacy

The information provided by the informants was reliable and adequate.

REASON FOR CONSULTATION

Patient came for pharmacological treatment and therapy.

CASE WORKED UP BY: Ms. Akshita Sharma (PDCCP Trainee MAN College, Guna)

SUPERVISED BY: Ms. Shivani Kirar (Assistant Professor & Clinical Psychologist MAN College, Guna)



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CHIEF COMPLAINTS

1. Getting negative thoughts repeatedly
 2. Having sexual thoughts
 3. Easily getting disturbed by a visual and thinking about it the entire day
 4. Constantly rubbing eyes
 5. Not swallowing the saliva to remove the thoughts
 6. Feeling stressed the entire day because of the thoughts
- } 4
Years

BRIEF HISTORY:

Index patient is a 19-year-old unmarried Hindu male, currently studying in the 12th standard, belonging to a middle socioeconomic background, and residing in Ruthiyai. He presented with complaints of recurrent intrusive negative and sexual thoughts, repeated eye rubbing, avoidance of swallowing saliva during negative thoughts, stress, restlessness, and difficulty controlling thoughts for the past four years. The symptoms reportedly began after a conflict involving his paternal uncle, following which he developed repetitive thoughts about harm and negative consequences befalling others. Over time, he started experiencing intrusive thoughts related to death, disturbing events, and sexual themes, which caused significant distress, guilt, and anxiety. To reduce the discomfort associated with these thoughts, he engaged in repeated eye rubbing and avoided swallowing saliva, both of which provided temporary relief. The symptoms gradually increased in frequency and intensity, leading to significant distress and adversely affecting his concentration, social interactions, and academic performance.



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TESTS ADMINISTERED:

1. Yale-Brown Obsessive Compulsive Scale (Y-BOCS)
2. Beck Depression Inventory (BDI)
3. Beck Anxiety Inventory (BAI)
4. Sacks Sentence Completion Test (SSCT)
5. Thematic Apperception Test (TAT)

BEHAVIOURAL OBSERVATION: The patient was well-groomed with maintained personal hygiene and entered the room voluntarily. He was calm during the administration of the test and maintained and made eye contact. He understood the instructions well. He was able to comprehend the questions well and didn't ask for any help during the entire administration. He maintained a willing and interested attitude towards the test and completed the tests in two sittings without any noted resistance.

YALE-BROWN OBSESSIVE COMPULSIVE SCALE (Y-BOCS)

RATIONALE FOR THE TEST

The test was conducted to assess the presence, nature, and severity of obsessive thoughts and compulsive behaviours in the patient. It helped in evaluating intrusive thoughts, repetitive behaviours, associated distress, degree of control, and the extent to which these symptoms interfere with daily functioning.

TEST RESULTS AND INTERPRETATION

A score of 33 on the Yale–Brown Obsessive Compulsive Scale (Y-BOCS) falls in the severe range of obsessive–compulsive symptoms. This indicates that the patient experiences persistent and distressing obsessions (repetitive intrusive negative and sexual thoughts, excessive preoccupation with harm befalling family members) along with compulsive behaviours (rubbing his eyes to “remove” thoughts and avoiding swallowing saliva due to the belief that negative thoughts may go inside him) that significantly interfere with daily functioning, cause marked anxiety, and consume considerable time.



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BECK DEPRESSION INVENTORY (BDI)**RATIONALE FOR THE TEST**

The test was conducted to evaluate the presence and severity of depressive symptoms, including feelings of guilt, hopelessness, low mood, and emotional distress, which may develop secondary to chronic obsessive thoughts and functional impairment. The assessment also helped in identifying comorbid depressive features commonly associated with obsessive-compulsive disorder.

TEST RESULTS AND INTERPRETATION

A score of **30 on the** Beck Depression Inventory (BDI) indicates **severe depressive symptoms**. This reflects marked emotional distress, guilt, negative thinking, and possible impairment in daily functioning, likely secondary to the chronic and distressing nature of obsessive thoughts and associated compulsions.

BECK ANXIETY INVENTORY (BAI)**RATIONALE FOR THE TEST**

The test was administered to assess the severity of anxiety symptoms such as excessive worry, restlessness, stress, and physiological tension experienced by the patient in response to intrusive thoughts. Since the patient reported persistent distress and difficulty controlling obsessive thoughts, assessment of anxiety was important to understand the extent of associated emotional disturbance.

TEST RESULTS AND INTERPRETATION

A score of **24 on the** Beck Anxiety Inventory (BAI) indicates **moderate level of anxiety**. This suggests that the patient experiences significant anxiety symptoms such as excessive worry, restlessness, tension, and stress associated with intrusive obsessive thoughts and compulsive behaviours.



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SACKS SENTENCE COMPLETION TEST (SSCT)**RATIONALE FOR THE TEST**

To assess patient's attitudes and feelings across key life areas, namely family, sex, interpersonal and self-concept, to better understand their underlying thoughts, emotions, and conflicts with respect to these areas, which aids to therapeutic planning.

TEST RESULTS AND INTERPRETATION

SUBTEST	SCORE	INTERPRETATION
1. Family Area	11	Mild disturbance
2. Sex Area	09	Mild disturbance
3. Interpersonal Relationships	20	Significant disturbance
4. Self-Concept	16	Significant disturbance

In the **family area** (overall score = 11), mild to moderate conflict is seen, particularly in relation to the mother (score = 5), indicating emotional dependency and unmet emotional needs. The patient has repeatedly expressed fear regarding harm or death occurring to family members and demonstrates excessive emotional attachment and anxiety concerning their safety.

In the **sexual area** (overall score = 9), mild conflict regarding females and heterosexual relationships is evident. This corroborates the patient's complaints of recurrent sexual thoughts about girls, associated guilt, disgust, and difficulty interacting comfortably with females or forming friendships with them.



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The greatest disturbance is observed in **interpersonal relationships** (overall score = 20), especially in the area of fear (score = 7). The findings of excessive sensitivity to disturbing situations, persistent anxiety, social discomfort, interpersonal mistrust, and difficulty coping with emotional stress justifies the score obtained. His tendency to develop grudges, overthink conflicts, and remain preoccupied with negative consequences also reflects interpersonal insecurity and heightened emotional reactivity.

In the **self-concept area** (overall score = 16), elevated scores indicate low self-esteem, uncertainty about personal abilities and future, along with guilt-proneness showing patient's repeated feelings of guilt and self-disgust associated with intrusive sexual and negative thoughts, as well as distress regarding his inability to control these thoughts.

Overall, the SSCT findings are suggestive of significant emotional distress, fearfulness, obsessive preoccupation, guilt, low self-worth, and interpersonal insecurity, which strongly corroborate the obsessive-compulsive, anxious, and depressive features elicited while history taking.

THEMATIC APPERCEPTION TEST (TAT)

RATIONALE FOR THE TEST

The Thematic Apperception Test (TAT) was administered to explore the patient's underlying emotional conflicts, unconscious fears, interpersonal perceptions, and dominant thought patterns. It was conducted to gain deeper insight into the patient's internal emotional functioning, coping mechanisms, unresolved conflicts, feelings of guilt, anxiety, and attitudes toward self and others, which may not be fully revealed through objective assessment measures.

TEST RESULTS AND INTERPRETATION

The patient's responses to the Thematic Apperception Test (TAT) reflected an emotionally vulnerable and internally conflicted personality organization characterised by themes of rejection, loneliness, emotional deprivation, guilt, and unfulfilled attachment needs. The narratives were predominantly reality-oriented, coherent, and structured, indicating preserved



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cognitive functioning and adequate reality testing; however, the emotional tone across stories was consistently melancholic, pessimistic, and emotionally burdened. Most stories revolved around interpersonal loss, betrayal, sacrifice, helplessness, and silent endurance of suffering. Romantic rejection, emotional abandonment, and inability to express feelings openly emerged repeatedly as central psychological concerns.

The stories were largely realistic, emotionally expressive, and complete, though many endings remained emotionally unresolved or pessimistic. The narratives frequently depicted emotionally painful situations involving rejection, failed love, domestic conflict, betrayal, loneliness, and helplessness. Fantasy elements were present mainly in the form of idealized love, longing, or escapist imagination rather than bizarre or psychotic content, suggesting neurotic-level conflict rather than severe thought disturbance. The language used was emotionally rich and introspective, with repeated emphasis on emotional pain, sacrifice, guilt, and isolation.

The patient predominantly identified with emotionally deprived, lonely, misunderstood, or rejected characters. Heroes were commonly adolescents or young adults experiencing emotional conflict, longing for affection, or inability to attain desired relationships. While male protagonists were more frequent and closely resemble the patient's age and emotional experiences, identification with suffering female characters was also observed, indicating projection of helplessness, emotional victimization, and suppressed dependency needs. The heroes were generally passive, emotionally sensitive, introspective, and unable to assert themselves effectively.

The dominant psychological needs emerging across stories included strong needs for affiliation, emotional attachment, nurturance, understanding, recognition, and acceptance. There was a persistent longing for reciprocal love and emotional validation. The stories also revealed needs for security, reassurance, and emotional closeness, alongside dependency wishes. Sexual themes were present but were associated with guilt, emotional conflict, and fear of rejection rather than healthy gratification. Achievement needs appeared secondary and were often connected to gaining affection or recognition from significant others. Underlying these themes was a strong need to avoid emotional pain and abandonment.



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The environment in most stories was perceived as emotionally depriving, rejecting, hostile, or unpredictable. Significant presses included rejection, loss, dominance, emotional aggression, lack of support, betrayal, and uncongenial interpersonal surroundings. Family and romantic relationships were frequently portrayed as emotionally unsafe or disappointing. Authority figures and social systems were often experienced as restrictive, dismissive, or controlling. Even when the physical environment was familiar, the emotional atmosphere remained cold, lonely, or threatening. The world was perceived as a place where emotional needs were rarely fulfilled and where suffering must often be silently tolerated.

The major themes emerging across the stories revolved around rejection, loneliness, emotional deprivation, betrayal, and unfulfilled attachment needs. Most narratives portray individuals who deeply desired affection, emotional closeness, understanding, and acceptance, yet repeatedly experience rejection, emotional distance, or disappointment in relationships. Themes of unrequited love, silent suffering, sacrifice for family expectations, helplessness, and fear of abandonment recurred consistently throughout the protocol. The stories frequently depicted characters who suppressed their emotions and tolerated distress passively rather than confronting conflicts directly. Romantic relationships were often idealized but ended in rejection, separation, or emotional dissatisfaction, reflecting insecurity and fear of emotional loss. Feelings of guilt, sadness, hopelessness, and emotional isolation were prominent, while happiness or fulfilment appeared temporary and fragile. Overall, the narratives suggested a strong longing for emotional security and interpersonal connection alongside persistent fears of rejection, loss, and emotional pain.

The dominant affects expressed were sadness, loneliness, guilt, fear, helplessness, hopelessness, longing, and emotional pain. Happiness or fulfilment was brief and often followed by rejection or disappointment. Emotional expression was largely internalized, with characters suppressing their distress rather than openly confronting conflict. Aggression was indirect and muted, often appearing as passive resentment, withdrawal, or emotional suffering rather than overt hostility.

The narratives revealed both intrapersonal and interpersonal conflicts. Intrapersonal conflicts primarily involved dependency versus fear of rejection, desired for intimacy versus fear of emotional pain, and personal wishes versus guilt or social expectations. Interpersonal conflicts



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were mostly related to romantic rejection, betrayal, family pressure, emotional neglect, and abusive relationships. Characters frequently suffering silently, withdraw emotionally, idealize lost relationships, or escaped into imagination rather than actively resolving problems. There was limited evidence of assertive or problem-focused coping. Emotional distress was managed through resignation, suppression, rationalization, fantasy, and withdrawal.

The predominant defence mechanisms appeared to be fantasy, withdrawal, repression, idealization, rationalization, denial, and suppression. Fantasy was used as an escaped from painful reality, especially in stories involving romance and longing. Withdrawal and emotional distancing were repeatedly used to cope with rejection and interpersonal pain. Rationalization was evident in accepting rejection as deserved or unavoidable. Repression and suppression were reflected in passive endurance of emotional suffering without direct confrontation.

The nature of anxiety across stories appeared primarily interpersonal and emotional. Anxiety stemmed from fear of rejection, abandonment, loneliness, emotional inadequacy, betrayal, and loss of affection. There was also evidence of guilt-related anxiety, especially around romantic and sexual themes. The emotional distress appeared chronic, internalized, and associated with feelings of helplessness and low self-worth.

Superego functioning appeared moderately harsh and guilt-inducing. Characters frequently sacrifice personal happiness for family expectations, tolerated mistreatment, or suppressed their emotional needs out of guilt, morality, or fear of hurting others. Romantic and sexual feelings were often associated with shame, self-blame, or feelings of unworthiness. Social and familial expectations strongly influence emotional functioning within the stories.

Ego functioning appeared fragile to moderate. The patient demonstrates intact reality testing, coherent thinking, and organized narrative production, suggesting preserved cognitive functioning. However, emotional integration appeared compromised due to excessive rumination, poor emotional regulation, dependency needs, and reliance on avoidant defences. The ego struggles to resolve conflicts adaptively and instead resorted to withdrawal, fantasy, and passive acceptance of distress.



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Overall, the TAT protocol reflects an emotionally sensitive and psychologically conflicted individual with strong dependency needs, fear of rejection, low self-esteem, and impaired coping with interpersonal stress. The narratives suggested significant emotional loneliness, unresolved attachment conflicts, guilt surrounding intimacy and sexuality, and a tendency toward internalization of distress. Defensive functioning was primarily avoidant and fantasy-based, while emotional suffering was expressed passively rather than directly. The findings were suggestive of underlying anxiety, depressive ideation, interpersonal insecurity, and obsessive preoccupation with emotional relationships and rejection.

Domains	Interpretation
Main Theme	Stories predominantly revolve around rejection, loneliness, emotional longing, betrayal, sacrifice, helplessness, and loss. Themes of unfulfilled love, emotional suffering, and silent endurance recur frequently.
Main Hero	Heroes are mostly emotionally sensitive, lonely, and rejected individuals, often adolescents or young adults. There is frequent identification with vulnerable and emotionally deprived characters.
Needs and Drives	Strong needs for affiliation, nurturance, emotional security, recognition, understanding, and love is evident. There is also a recurring desire for acceptance and emotional reassurance.
Environment	The environment is largely perceived as emotionally unsafe, rejecting, restrictive, hostile, or unpredictable. Relationships are often marked by betrayal, lack of support, or emotional distance.
Conception of Environment	The world is viewed as unfair, emotionally depriving, and difficult to trust. Interpersonal relationships are seen as unstable and painful.



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Figures Seen As	Characters are viewed as emotionally burdened, helpless, lonely, submissive, or betrayed. Many figures silently tolerate suffering rather than confronting it.
Significant Conflicts	Major conflicts include love versus rejection, personal desires versus family/social expectations, dependency versus loneliness, and emotional expression versus suppression.
Nature of Anxiety	Anxiety appears related to rejection, abandonment, loneliness, emotional loss, guilt, and fear of not being accepted or valued by others.
Main Defences	Predominant defences include fantasy, withdrawal, idealization, repression, denial, suppression, rationalization, and passive avoidance.
Superego	The superego appears harsh and guilt-inducing at times, especially regarding relationships, duty, and self-worth. Social and family expectations strongly influence the narratives.
Manifestation	Emotional distress is expressed through sadness, loneliness, passive suffering, emotional withdrawal, hopelessness, and self-devaluation rather than direct aggression.
Ego Integration	Ego functioning appears fragile to moderate. The patient retains reality contact and insight, but struggles with emotional regulation, assertiveness, coping with rejection, and adaptive conflict resolution.



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The TAT narratives reveal a pattern of emotional vulnerability, fear of rejection, longing for affection, and difficulty coping with interpersonal loss or disappointment. The patient identifies strongly with lonely and emotionally deprived characters who suppress their feelings and silently endure suffering. Relationships are idealized yet experienced as disappointing, rejecting, or unattainable. Defensive functioning is largely avoidant and fantasy-based, suggesting difficulty expressing emotions directly and managing emotional conflict adaptively. The stories also indicate low self-worth, sensitivity to criticism or abandonment, and dependence on emotional validation from others. Overall, the narratives suggest underlying insecurity, emotional loneliness, depressive ideation, and impaired coping with interpersonal stressors.

OVERALL IMPRESSION

The overall psychological profile suggests a young male experiencing severe obsessive–compulsive symptoms accompanied by significant anxiety, depressive features, interpersonal insecurity, and low self-esteem. The clinical history and assessment findings indicate persistent intrusive thoughts related to harm, death, and sexuality, which are experienced as distressing, unwanted, and difficult to control. These obsessions are maintained through compulsive behaviours such as repeated eye rubbing and avoidance of swallowing saliva, which provide temporary relief but reinforce the obsessive–compulsive cycle.

The patient demonstrates intact reality testing and adequate cognitive functioning; however, his thinking is frequently influenced by excessive guilt, heightened responsibility, catastrophic interpretations of thoughts, and difficulty tolerating uncertainty. Assessment findings reveal moderate anxiety and severe depressive symptoms characterized by persistent distress, worry, guilt, sadness, hopelessness, and emotional withdrawal. He appears highly sensitive to emotional experiences and tends to overthink and ruminate about distressing events and interpersonal situations.

Interpersonally, the patient exhibits insecurity, fear of rejection, discomfort in interactions with females, and a strong need for acceptance and emotional validation. Projective assessment findings reveal recurring themes of loneliness, rejection, emotional deprivation, helplessness, and unfulfilled attachment needs. He tends to internalize distress, idealize relationships, and



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cope with emotional conflicts through avoidance, fantasy, withdrawal, suppression, and rumination rather than direct problem-solving.

Overall, the findings are suggestive of an individual with severe OCD, moderate anxiety, severe depressive features, interpersonal sensitivity, and maladaptive coping strategies. Despite significant psychological distress, the patient demonstrates good insight, motivation for improvement, and adequate capacity to benefit from structured psychological interventions such as CBT and Exposure and Response Prevention (ERP), along with appropriate psychiatric management and family support.

RECOMMENDATIONS AND SUGGESTIONS

1. Pharmacotherapy:

A psychiatric consultation is recommended for initiation and management of medication, particularly SSRIs, considering the severity of obsessive-compulsive symptoms along with associated anxiety and depressive features. Regular follow-up should be maintained to monitor improvement, side effects, and treatment adherence.

2. Exposure and Response Prevention (ERP)

It is likely to benefit the patient by helping him gradually face intrusive negative and sexual thoughts without engaging in compulsive behaviours such as eye rubbing and avoiding swallowing saliva. Repeated exposure to these thoughts while preventing ritualistic responses would reduce anxiety, weaken maladaptive beliefs about the dangerousness of thoughts, increase tolerance of uncertainty, and ultimately decrease the frequency and distress associated with obsessive thoughts and compulsive behaviors.

3. Psychoeducation:

The patient and family should be educated regarding the nature of OCD, intrusive thoughts, compulsions, anxiety, and associated depressive symptoms. It should be emphasized that intrusive thoughts are involuntary and do not reflect actual intentions or moral character.



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4. Anxiety and Stress Management:

Relaxation techniques such as deep breathing exercises, grounding techniques, mindfulness, and progressive muscle relaxation may help reduce anxiety and emotional distress. Stress management training should also be incorporated to improve overall emotional functioning.

5. Social Skills and Interpersonal Intervention:

Therapeutic intervention should address interpersonal insecurity, fear of rejection, and discomfort in interactions with females. Improving communication skills, self-confidence, and social engagement may help reduce isolation and enhance interpersonal functioning.

6. Regular Follow-up:

Regular psychiatric and psychological follow-up is essential to monitor symptom progression, assess treatment response, prevent relapse, and ensure continuity of care.

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